

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: JBPeriod: July 2 to September 30, 2024Report Date: October 1, 2024Coverage: Bed: 1772/2174h (81.5%)Chair: 1474/2174h (67.8%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 8 PM to 6 AM		Low HR 9 PM to 11 PM		Very High HR 8 PM to 6 AM	
Vital Stat		Avg		Min	
Heart Rate		78 bpm		40 bpm	
Breathing		15 brpm		11 brpm	

Last 24 hours: three low heart rate episodes in the evening; one very high heart rate episode in the evening.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
8 PM to 6 AM	14h	4	Elevated Breathing	15	11	31	Check SpO2 and lung sounds; assess for fluid overload or early infection
9 PM to 11 PM	4h	3	Low Heart Rate (brief)	44	40	50	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
8 PM to 6 AM	10h	1	Very High Heart Rate (brief)	139	71	145	Check temp, hydration, pain; review for arrhythmia or infection

Patient clinical status over 91 days from Jul 02 to Sep 30

HR

Breathing

Elevated HR Jul 02 to Jul 12 (11d)	Elevated HR Sep 20 to Sep 24 (5d)	Elevated Breathing Jul 15 to Jul 24 (10d)	High HR Jul 08 to Jul 12 (5d)	High HR Sep 14 to Sep 19 (6d)	High HR Sep 23 to Sep 26 (4d)	Low HR Jul 13 to Sep 05 (55d)	Low HR Sep 08 to Sep 21 (14d)	Low HR Sep 24 to Sep 30 (7d)	Very High HR Sep 10 to Sep 11 (2d)	Very High HR Sep 21 to Sep 29 (9d)
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291 episodic events Detected, spanning 26 days total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
291	624h	Low Heart Rate	3	100%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 10	7 AM to 1 PM	35h	5	Very High Heart Rate	141	74	148	Check temp, hydration, pain; review for arrhythmia or infection
Sep 21	3 AM to 8 AM	66h	9	Very High Heart Rate	137	68	145	Check temp, hydration, pain; review for arrhythmia or infection
Jul 08	12 AM to 1 AM	7h	6	High Heart Rate	104	45	127	Check temp, hydration, pain; review for arrhythmia or infection
Sep 14	5 AM to 6 AM	26h	14	High Heart Rate	105	51	130	Check temp, hydration, pain; review for arrhythmia or infection
Sep 23	6 PM to 8 PM	7h	6	High Heart Rate	105	55	145	Check temp, hydration, pain; review for arrhythmia or infection
Jul 13	10 PM to 11 PM	339h	155	Low Heart Rate	43	40	74	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jul 07	8 AM to 10 AM	20h	12	High Breathing (brief)	33	15	55	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

+ 5 additional condition(s) across Elevated Breathing, Elevated Heart Rate, Low Heart Rate; details in monitoring data.

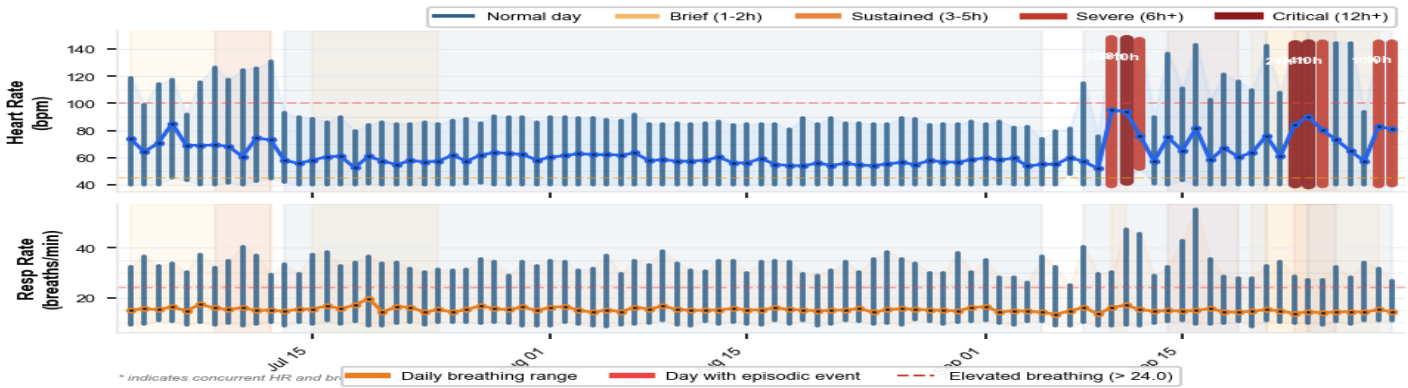
The P5 to P95 heart rate spread was 54 bpm (43 to 97), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Very High Heart Rate (avg 137 bpm, peak 145 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Heart Rate (avg 98 bpm, peak 131 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Heart Rate (avg 105 bpm, peak 130 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 27 brpm, peak 36 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection

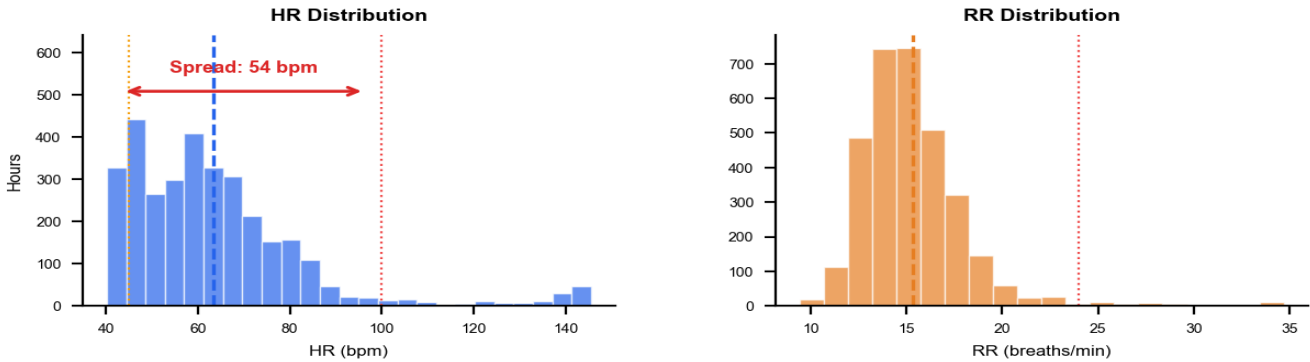
Clinical Pattern Observations:

- 1. **Sustained finding:** 24h continuous Very High Heart Rate beginning Sep 23.
- 2. **High variability:** HR spread 54 bpm (42 to 96).
- 3. **Clustered pattern:** Episodic events on 8 of 91 days (8%).

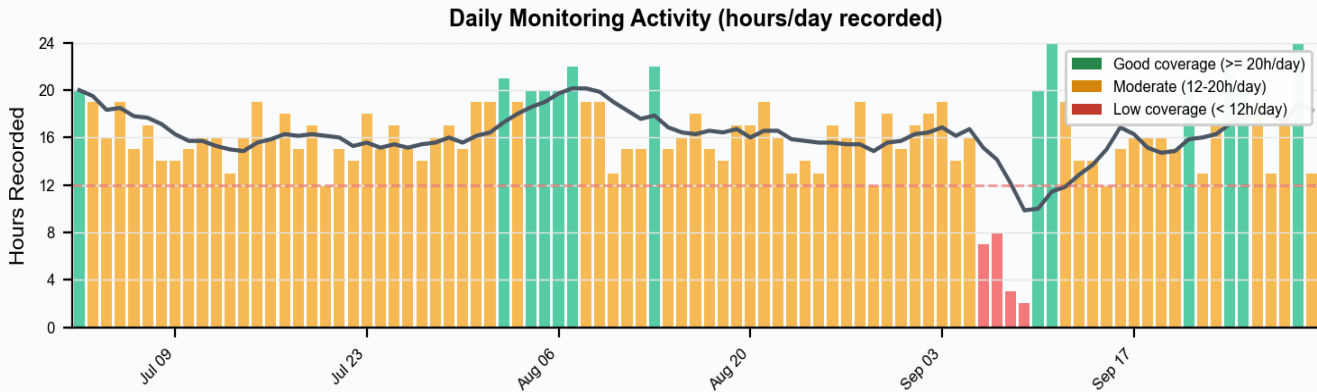


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.